SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Lutetium-177 oxodotreotide (Lutathera / PRRT)

Progressive, unresectable or metastatic, well-differentiated G1/G2 somatostatin-receptor-positive gastroenteropancreatic NET under NCCP 00642a. · NCCP 00642 · version 2 · 56-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
PRRT carries radiation to somatostatin receptors on NET cells to help control disease that cannot be removed or has spread. A specialist NET and nuclear-medicine team checks receptor imaging, organ function and your priorities before recommending it.

Schedule
Four administrations are planned, usually eight weeks apart. Lutathera is infused over about 30–40 minutes, with a four-hour kidney-protective amino-acid infusion and anti-sickness treatment. The visit is much longer than the Lutathera infusion; an overnight stay may be needed.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Lutetium-177 oxodotreotide
RECEPTOR-TARGETED RADIOPHARMACEUTICAL

- ☐ Nausea, vomiting, appetite loss and fatigue
- ☐ Low blood counts, infection and bleeding
- ☐ Kidney and liver injury
- ☐ Hormone crisis and tumour lysis syndrome
- ☐ Infusion allergy
- ☐ Delayed marrow disorders including MDS and acute leukaemia
- ☐ Fertility effects, pregnancy harm and radiation exposure to others

Lysine / arginine amino acids
SUPPORTIVE INFUSION

- ☐ Nausea / vomiting
- ☐ Raised potassium
- ☐ Fluid overload in susceptible patients

Discuss the exact SSTR-positive NET indication, benefit and uncertainty, alternatives, kidney and marrow injury including delayed MDS/leukaemia, hormone crisis, tumour lysis, fertility/pregnancy, radiation precautions and the monitoring and SSA plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00642 v2.

SACTCheck v0.84.37
Developed by Dr Paul O'Brien